DOC_ID: SAFETY_DOC_007
TITLE: CardioZen and Serum Potassium Monitoring in Patients with Chronic Kidney Disease
PUBLISHER: HealthOne Group Medication Safety Committee
PUBLICATION_DATE: 2026-03-01
STATUS: ACTIVE

HEALTHONE GROUP - MEDICATION SAFETY COMMUNICATION

Document ID: SAFETY_DOC_007
Issued: 1 March 2026
Classification: Safety Communication - Action Required

| SYNTHETIC DOCUMENT - NOT REAL MEDICAL GUIDANCE. This document is
| fabricated for the purposes of a software demonstration. CardioZen is an
| invented medication that does not exist. Nothing in this document describes
| any real product, and nothing in it should be used to inform any clinical
| decision.

1. Purpose

This communication informs prescribers, pharmacists and clinical review staff of a monitoring requirement applying to patients treated with CardioZen who also carry a diagnosis of chronic kidney disease. It supersedes no previous communication. It takes effect on the date of issue.

2. Background

CardioZen is cleared substantially by renal excretion. In patients whose kidney function is already reduced, clearance is slower and plasma concentrations accumulate over a period of weeks. The principal consequence of accumulation is a rise in serum potassium.

Post-marketing surveillance across the HealthOne network identified eleven cases over the preceding eighteen months in which potassium elevation was identified only after the patient became symptomatic. In nine of those eleven cases, no potassium measurement had been recorded in the preceding six months. The Committee's view is that the interval between measurements, rather than the availability of the measurement itself, was the contributing factor.

3. Monitoring requirement

For any patient receiving CardioZen who has a recorded diagnosis of chronic kidney disease, serum potassium should be checked at least every three months for the duration of therapy.

The requirement applies from the date of this communication. It applies to patients already established on CardioZen as well as to new starts. Where a patient has had no potassium measurement within the stated interval, the responsible clinical team should arrange one.

4. Population not covered by this requirement

Patients receiving CardioZen who do not carry a diagnosis of chronic kidney disease are outside the scope of section 3. The Committee considered whether to extend the requirement to all CardioZen recipients and concluded that the evidence does not presently support it.

5. Reporting

Suspected adverse reactions should be reported through the usual HealthOne pharmacovigilance channel. Reporting is not a substitute for the monitoring described in section 3.

6. Review

This communication will be reviewed within twelve months of issue, or earlier if surveillance data warrant it.

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